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Primary Research Dossier

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Project	ClearPath — AI-Powered Mental Health Triage and Navigation
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Document purpose. This dossier provides the foundational research layer underpinning the ClearPath product. It establishes the problem, synthesizes the evidence base, and frames the opportunity. Product decisions, design choices, and technical tradeoffs documented in the companion Product Dossier and SRS are traceable back to findings in this document. All claims carry source attribution, reliability ratings, and limitation notes where material.

Table of Contents

- 1. Research Methodology
- 2. The Core Problem: Intent-to-Care Gap
- 3. Epidemiology: Scale of Mental Health Need
- 4. Workforce Supply: A Structural Shortage
- 5. Access Barriers: Where the System Fails
- 6. Economic Impact: The Cost of Inaction
- 7. Digital Mental Health: Evidence for Technology Interventions
- 8. Market Context
- 9. Regulatory and Ethical Landscape
- 10. Competitive Landscape Analysis
- 11. Key Research Conclusions
- 12. References

1. Research Methodology

1.1 Scope

This research covers the United States mental healthcare access landscape as of 2024–2026. The scope is bounded to:

- Epidemiology of mental illness prevalence and treatment access
- Healthcare workforce supply and shortage projections
- Access barriers including wait times, insurance complexity, and cost
- Economic burden of untreated mental illness, particularly in the workplace
- Evidence for digital and AI-enabled mental health interventions
- Regulatory environment for digital health tools
- Competitive product landscape

1.2 Sources Used

Research draws from five tiers of sources, prioritized by reliability:

Tier	Source Type	Reliability	Examples
1 — Primary Federal	Federal agencies, government surveys	High	SAMHSA NSDUH, HRSA Workforce Data, CDC/NCHS, FDA Guidance
2 — Peer-Reviewed	Academic journals, RCTs, systematic reviews	High	BMJ Open, Psychiatric Services, Nature Communications Medicine, PMC/NCBI
3 — Policy/Advocacy	Nonpartisan policy institutes, professional bodies	Moderate-High	Commonwealth Fund, APA, NAMI, Gallup, IFEBP
4 — Market Research	Commercial market-sizing firms	Moderate	SNS Insider, Mordor Intelligence, Straits Research
5 — Industry/Vendor	Proprietary studies, vendor reports	Moderate (directional only)	Wysa, Woebot, Big Health

1.3 Reliability Conventions

Throughout this document, reliability labels are applied as follows:

- **[High]** — Federal primary source or peer-reviewed publication; methodology is documented and reproducible
- **[Moderate-High]** — Professional body or nonpartisan policy institute citing primary data; methodology partially documented
- **[Moderate]** — Advocacy, commercial, or vendor synthesis; directional use only; noted where estimates vary materially across firms
- **[Directional only]** — Proprietary, non-peer-reviewed, or self-reported industry data; cited for illustration, not as definitive

1.4 Verification Process

Key statistics were cross-checked against primary federal and peer-reviewed sources before inclusion. Figures are attributed to the primary source even when first encountered via a secondary synthesis. Where a statistic is commonly misquoted in industry summaries, the body text states the precise definition, scope, and limitation inline — for example, wait-time benchmarks scoped to community mental health centers, or economic figures distinguished by component and time horizon.

2. The Core Problem: Intent-to-Care Gap

2.1 Framing

The critical failure in U.S. mental healthcare is not a lack of willingness to seek help — it is the system's structural inability to convert that intent into care quickly and correctly.

When a motivated adult decides to seek mental health support today, they face a sequence of friction points that — individually — might be surmountable, but together create a systemic failure:

- 1. **No structured self-assessment** — there is no validated, accessible tool to help them understand what type of care they actually need before they begin searching
- 2. **Invisible availability** — directories show who exists, not who is accepting new patients
- 3. **Insurance opacity** — coverage is often unknown until after the first visit
- 4. **Long waits** — even when a suitable provider is found, waiting weeks to months is standard
- 5. **No step-down logic** — the system defaults to weekly individual therapy regardless of clinical need, bypassing evidence-based stepped-care models

The outcome: people either never enter care, enter mismatched care, or abandon the process entirely after weeks of fruitless effort.

2.2 Primary User Profile

The target user is not the most acutely ill — that population is served (imperfectly) by crisis systems. The most underserved population is the large, commercially accessible middle:

Attribute	Profile
Age range	25–45 years
Employment status	Employed, often with employer-sponsored insurance
Digital literacy	Moderate to high
Clinical state	Mild-to-moderate anxiety or depression; not in acute crisis
Behavioral pattern	Has searched for help; hit walls of unavailability; stalled
Primary need	Fast, personalized, actionable guidance — not generic information

Why this user: They are large in number (tens of millions), commercially reachable through employer benefit channels, capable of completing a digital intake, and most harmed by the intent-to-care gap. Their treatment gap drives the largest share of measurable economic loss.

3. Epidemiology: Scale of Mental Health Need

3.1 Prevalence of Mental Illness

Finding: In 2024, approximately **61.5 million U.S. adults (23.4%)** met criteria for any mental illness (AMI). Of these, only **52.1% received any mental health treatment** in the past year, leaving approximately **29.5 million adults (48%) with no treatment** despite meeting diagnostic criteria. **[High]**

Source: Substance Abuse and Mental Health Services Administration (SAMHSA). (2025). *Key Substance Use and Mental Health Indicators in the United States: Results from the 2024 National Survey on Drug Use and Health (NSDUH)*. [1]

Limitation: Self-report; stigma likely causes underreporting. Cross-sectional design captures prevalence at a point in time. Figures match SAMHSA published tables (61.5 million / 23.4%).

3.2 Substance Use Disorders

Finding: The treatment gap is even wider for substance use disorders: approximately **80% of people** who needed treatment in 2024 did not receive it. **[High]**

Source: SAMHSA NSDUH 2024 [1, 4]

Limitation: Same survey caveats as §3.1 above.

3.3 Depression and Income Disparity

Finding: Federal data consistently show depression prevalence is substantially higher among lower-income adults than higher-income adults across survey cycles. **[High]**

Source: CDC/NCHS data briefs, multiple cycles

Note: The specific income-to-prevalence multiplier varies by survey year and design. This is stated as a directional finding rather than a fixed ratio; citing a specific multiplier from any single cycle risks false precision.

3.4 Summary Table

Metric	Value	Year	Source
U.S. adults with any mental illness	61.5 million (23.4%)	2024	SAMHSA NSDUH [1, 4]
Of those, received no treatment	~48% (~29.5 million)	2024	SAMHSA NSDUH [1, 4]
SUD treatment gap	~80%	2024	SAMHSA NSDUH [1]

4. Workforce Supply: A Structural Shortage

4.1 Geographic Access Shortage

Finding: As of **December 2, 2025, 40% of the U.S. population — approximately 137 million people —** lives in a federally designated Mental Health Professional Shortage Area (Mental Health HPSA). **[High]**

Source: HRSA, National Center for Health Workforce Analysis. (December 2025). *Behavioral Health Workforce Brief 2025*. [9]

Note: HPSA designations are updated regularly; this figure reflects the December 2, 2025 data snapshot published in the brief.

4.2 Psychiatrist Supply

Finding: There were **52,164 psychiatrists** in the United States as of 2023. Federal workforce projection models indicate substantial shortages across multiple behavioral health professions are projected to persist through the 2030s. **[High]**

Source: HRSA Behavioral Health Workforce Brief 2025 [9]

Limitation: Supply-demand modeling involves assumptions about utilization rates, population growth, and provider productivity; projections carry inherent uncertainty.

4.3 Psychologist Availability

Finding: A majority of psychologists surveyed by the American Psychological Association report **not accepting new patients**. Among those with active waitlists, average waits of **three months or longer** are common. **[Moderate-High]**

Source: American Psychological Association practitioner workforce surveys, 2023 [20]

Limitation: Self-selected respondent pool; may overrepresent busier practitioners. Results are from a 2023 survey and may not reflect 2025–2026 conditions precisely.

4.4 Summary Table

Metric	Value	Year	Source
Population in Mental Health HPSA	40% (137 million)	Dec 2, 2025	HRSA Workforce Brief 2025 [9]
Total U.S. psychiatrists	52,164	2023	HRSA Workforce Brief 2025 [9]
Psychologists not accepting new patients	Majority	2023	APA practitioner survey [20]

5. Access Barriers: Where the System Fails

5.1 Wait Times

Finding — Community Mental Health Centers: The national average wait between a client's first outreach or referral and their first appointment at **community mental health centers** is **48 days**, according to an MTM Services analysis of approximately 10,000 care-access flowcharts from approximately 1,000 centers across 47 states, cited by the National Council for Mental Wellbeing. **[Moderate-High]**

Source: National Council for Mental Wellbeing / MTM Services. *CCBHC Impact Report* [16]

Important scope note: This figure is specific to community mental health centers and the 2021 analysis period. It is the most-cited national wait-time benchmark but should not be generalized as a universal behavioral health appointment average across all care settings or years.

Finding — Patient-Reported Waits: A peer-reviewed cross-sectional survey found mean reported wait times of **94.1 days**, with **85.2% of respondents** characterizing their wait as too long. **[High]**

Source: Subotic-Kerry, M. et al. (2025). *While they wait: a cross-sectional survey on wait times for mental health services*. *BMJ Open*, 15(3), e087342. [18]

Limitation: International sample; U.S.-specific figures may differ from the aggregate mean.

Finding — Psychiatrist Availability: A 2022 mystery-shopper study found only approximately **18.5% of psychiatrists** available to new patients in a five-state sample; the median in-person wait was **67 days** and the median telepsychiatry wait was **43 days**. **[High]**

Source: Mullangi, S. et al. (2023). *Low availability, long wait times, and high geographic disparity of psychiatric outpatient care in the US*. Psychiatric Services. [21]

Limitation: Five-state sample; 2022 data; may not fully represent national conditions.

5.2 Insurance and Cost Barriers

Finding — Cost: Roughly **1 in 5 Americans** delayed or did not receive mental health care due to cost. **[Moderate-High]**

Source: Commonwealth Fund. (November 2024). *State of Health Insurance Coverage in the U.S.: 2024 Biennial Survey*. [10]

Finding — In-Network Access: About **1 in 3 people** with private insurance reported difficulty finding an in-network mental health therapist and were more likely to seek out-of-network care compared to medical/surgical care. **[Moderate]**

Source: NAMI survey data, via insurance/MHPAEA analysis [15]

Limitation: Advocacy survey; methodology not fully detailed; exact ratio varies by source year.

Finding — Parity Gaps: Out-of-network utilization for behavioral health is materially higher than for medical/surgical care. Mental health parity enforcement remains variable across states and federally. **[High]**

Source: Commonwealth Fund [11, 14]; parity synthesis [13]

5.3 Structural Failures in the Current System

Failure Mode	Description
Unstructured intake	Patients self-refer with no validated symptom data; clinicians re-collect what PHQ-9/GAD-7 capture in minutes
Undirected matching	Directories match on keyword and geography, not clinical severity or specialty need
Invisible wait times	Little real-time availability data; users apply in parallel with no feedback loop

Failure Mode	Description
Insurance opacity	Coverage often unknown until after the first visit
No stepped-care logic	Defaults to weekly individual therapy regardless of need, bypassing evidence-based stepped-care models
Passive crisis handling	Most tools display a hotline number; few implement active triage protocols

5.4 Summary Table

Metric	Value	Setting/Notes	Source
Average first-outreach-to-appointment wait	48 days	Community mental health centers (2021)	National Council / MTM [16]
Patient-reported mean wait	94.1 days	Cross-sectional survey; international sample	Subotic-Kerry et al. 2025 [18]
Psychiatrists accepting new patients	~18.5%	Five-state mystery-shopper, 2022	Mullangi et al. 2023 [21]
Median in-person psychiatry wait	67 days	Same study	Mullangi et al. 2023 [21]
Delayed care due to cost	~1 in 5 Americans	Commonwealth Fund 2024	[10]
Difficulty finding in-network therapist	~1 in 3 privately insured	NAMI survey	[15]

6. Economic Impact: The Cost of Inaction

6.1 Total Avoidable Cost of Mental Health Inequities

Finding: Mental health inequities in the United States cost an estimated **\$477.5 billion in avoidable or unnecessary expenditures in 2024**, spanning chronic physical health conditions, emergency department overutilization, productivity loss, and premature death. Annual costs are projected to reach approximately **\$1.3 trillion by 2040**, with cumulative costs from 2024–2040 approaching **\$14 trillion**.
[Moderate-High]

Source: Deloitte Health Equity Institute & Meharry School of Global Health (Dawes, D. et al.). (May 2024). *The Projected Costs and Economic Impact of Mental Health Inequities in the United States*. [27]

Critical nuance: The \$477.5 billion figure represents the **total avoidable cost of mental health inequities** — it is not lost productivity alone. Of that total, productivity loss accounts for approximately **\$116 billion**. The \$14 trillion figure is a **cumulative** projection over 2024–2040, not an annual figure.

6.2 Workplace Absence Cost

Finding: Unplanned absences attributable to poor employee mental health cost the U.S. economy an estimated **\$47.6 billion annually**. **[Moderate-High]**

Source: Gallup. (2023, updated 2025). *The Economic Cost of Poor Employee Mental Health*. Sample size approximately 15,000+. [26]

Limitation: Self-reported; absence attribution methodology is subject to recall bias.

6.3 Depression and Lost Productive Time

Finding: Depression alone is associated with approximately **\$44 billion per year** in lost productive time among U.S. workers. **[Moderate]**

Source: Industry synthesis citing Stewart et al. and related research, 2024 [29]

Limitation: Derived from cost-of-illness modeling; productivity valuation methodologies vary.

6.4 Employer Awareness Gap

Finding: Roughly **half of employees** report not knowing how to access mental health care through their employer. **[Moderate]**

Source: Workplace mental health survey synthesis (NAMI/Gallup/APA/Mind Share Partners), 2026 [25]

Limitation: Self-reported; composite from multiple surveys with differing samples and years.

6.5 Summary Table

Economic Metric	Value	Notes	Source
Avoidable cost of mental health inequities (U.S., 2024)	\$477.5 billion	Total — includes chronic conditions, ED overuse, productivity loss, premature death	Deloitte/Meharry [27]
Of that: productivity loss component	~\$116 billion	Subset, not the whole	Deloitte/Meharry [27]
Projected annual cost (mental health inequities, 2040)	~\$1.3 trillion	Annual estimate	Deloitte/Meharry [27]

Economic Metric	Value	Notes	Source
Projected cumulative cost (2024–2040)	~\$14 trillion	Multi-year cumulative	Deloitte/Meharry [27]
Absence costs from poor mental health	~\$47.6 billion/year	Employer perspective	Gallup [26]
Lost productive time from depression	~\$44 billion/year	Worker perspective	Industry synthesis [29]

7. Digital Mental Health: Evidence for Technology Interventions

7.1 Generative AI and CBT Engagement

Finding: A 2026 randomized controlled trial (N=540) found that a generative-AI-enabled cognitive behavioral therapy (CBT) application significantly enhanced participant engagement compared to digital workbooks among adults with elevated anxiety or depression symptoms. **[High]**

Source: McFadyen et al. (January 15, 2026). *Increasing engagement with CBT using generative AI: a randomized controlled trial*. Communications Medicine (Nature Portfolio). [35]

Limitation: Open-label design; outcomes measured by self-report; single-country sample; generalizability to other settings requires confirmation.

7.2 AI-Supported CBT vs. Face-to-Face

Finding: A 2024–2025 comparative study found AI-supported digital CBT showed **comparable effectiveness** to face-to-face CBT for generalized anxiety, while requiring substantially less clinician time per patient. **[High]**

Source: PMC/NCBI. (2024–2025). *Combining Artificial Intelligence and Human Support in Mental Health*. PMC12117275. [34]

Limitation: Propensity-matching design, not a fully randomized head-to-head trial; self-selection effects possible.

7.3 Early AI Companion Evidence

Finding: Woebot's foundational randomized controlled trial demonstrated statistically significant reduction in PHQ-9 depression scores within **two weeks** of use among college students with anxiety and depression. **[Moderate-High (directional)]**

Source: Industry summary of the Woebot RCT results, citing a published peer-reviewed trial [33]

Limitation: Original RCT conducted in a specific population (college students); generalizability to working adults is not established by this study alone.

7.4 AI Crisis Detection

Finding: Wysa, a conversational AI mental health platform, reported that its AI system flagged **82% of crisis incidents** compared to only 18% identified through user self-report, across a dataset of approximately 19,000 users. **[Directional only]**

Source: Wysa / BusinessWire. (April 15, 2024). Business wire press release. [55]

Limitation: Proprietary, non-peer-reviewed data; possible selection bias; not independently validated. This figure is cited directionally to support the feasibility of AI-based crisis detection, not as a precision benchmark.

7.5 Evidence Quality Summary

Intervention	Evidence Level	Finding
Generative AI + CBT (engagement)	RCT — High	Significant improvement in engagement vs. digital workbooks
AI-supported vs. face-to-face CBT	Comparative study — High	Comparable effectiveness; lower clinician time
AI chatbot (PHQ-9 reduction)	RCT (specific population) — Moderate-High	Significant PHQ-9 reduction within 2 weeks
AI crisis detection	Proprietary — Directional only	82% crisis flag rate claimed; not peer-reviewed

8. Market Context

8.1 Market Size

Finding: The U.S. mental health apps market was valued at approximately **\$3.87 billion in 2025** and is projected to reach approximately **\$18.14 billion by 2035**, representing a CAGR of approximately 16.8%. The global market is projected at approximately **\$45.12 billion by 2035**. **[Moderate]**

Source: SNS Insider (via GlobeNewswire/Yahoo Finance). (February 2026). [40]

Important note: Market-sizing estimates vary materially across research firms. At least one other firm estimates the U.S. market at approximately \$2.31 billion in 2025. These figures are directional; significant variance in methodology and market definition drives the divergence.

8.2 Employer Market Dynamics

Finding: Employer investment in digital mental health benefits has been **expanding year over year**. More employers are offering virtual/EAP mental health resources, and more are gaining access to utilization data. [Moderate]

Source: International Foundation of Employee Benefit Plans. (2024). *Mental Health and Substance Use Disorder Benefits: 2024 Survey*. [30]

Finding: North America is the largest regional segment of the digital mental health market, with enterprise growth driven by **self-insured employers** adopting workforce mental health platforms. [Moderate]

Source: Mordor Intelligence. (2026). *Mental Health Apps Market Report*. [43]

8.3 Mid-Market Gap

Research synthesis indicates a measurable gap at the **mid-market employer tier** (200–5,000 employees):

- Large enterprise platforms (e.g., Spring Health, Lyra) are clinically validated but cost-structured for Fortune 500 employers
- Teletherapy marketplaces (e.g., BetterHelp, Talkspace) operate in closed ecosystems and do not connect users to external or in-person providers
- AI wellness companions (e.g., Woebot, Wysa) focus on ongoing support, not triage and navigation
- Provider directories (Psychology Today, Zocdoc) offer large databases but no validated clinical intake or severity-matched routing

9. Regulatory and Ethical Landscape

9.1 FDA Regulatory Framework

Finding: The FDA's **General Wellness Policy for Low Risk Devices** (2020) provides a relevant framework for digital health tools that promote wellness generally and are not intended to treat, diagnose, cure, or mitigate a specific disease. [High — Primary regulatory source]

Source: FDA. (September 27, 2020). *General Wellness: Policy for Low Risk Devices*. [54]

Application: A triage and navigation tool using validated screening instruments (PHQ-9/GAD-7) — without making diagnostic claims — is intended to be positioned within this framework. A formal regulatory assessment must be completed prior to launch.

Finding: Mounting regulatory and legal pressure exists for AI mental health tools, with increasing FDA scrutiny of products making clinical claims beyond general wellness. **[Moderate-High]**

Source: Gardner Law. (September 25, 2025). *AI Mental Health Tools Face Mounting Regulatory and Legal Pressure*. [52]; Sidley Austin LLP. (2026). *U.S. FDA and CMS Actions on Generative AI-Enabled Mental Health Devices*. [51]

9.2 HIPAA Considerations

For digital health products:

- A product that stores no personally identifiable health information and has no covered-entity relationship has limited HIPAA obligations in prototype/MVP form
- A formal legal review is required before any pilot involving identified employee data
- A HIPAA-compliant architecture (including Business Associate Agreements) is a prerequisite for employer or insurer pilots with identified user data

9.3 Mental Health Parity

Finding: The Mental Health Parity and Addiction Equity Act (MHPAEA) requires that mental health and SUD benefits not be subject to more restrictive limitations than medical/surgical benefits. Enforcement remains variable at the state level and federal rulemaking is ongoing. Payers need demonstrable triage infrastructure to support parity compliance. **[High]**

Source: Commonwealth Fund [11, 14]; parity synthesis [13]

9.4 Key Ethical Considerations

Consideration	Research Finding	Implication
Scope of practice	PHQ-9 and GAD-7 are validated screening tools, not diagnostic instruments	Product must make no diagnostic claims; defer interpretation to licensed providers
Algorithmic bias	Matching may underserve rural ZIPs, Medicaid enrollees, non-English speakers	Require "no matches" fallback; plan demographic-parity audits
Stigma and language	Validated questionnaire wording reduces stigma vs. open free-text requests	Use verbatim validated instrument wording; avoid clinical jargon

Consideration	Research Finding	Implication
Informed consent	User must understand scope, data use, and limitations before beginning	Clear intake disclaimers required

10. Competitive Landscape Analysis

10.1 Category Map

Category	Representative Products	Core Strength	Critical Gap
Provider directories	Psychology Today, Zocdoc	Large, searchable databases	No validated intake; no clinical severity matching
Teletherapy marketplaces	BetterHelp, Talkspace	Scale, brand recognition, accessibility	Closed ecosystems; no routing to external or in-person providers
Enterprise workforce platforms	Spring Health, Lyra Health	Clinically validated; enterprise-grade infrastructure	Higher cost structure; designed for large employers (5,000+)
AI companions	Woebot, Wysa	Clinically studied AI for ongoing support	Ongoing support focus, not triage and navigation
Wellness / meditation	Headspace, Calm	Strong B2B distribution base	Wellness, not clinical-severity navigation
EAP platforms	Various	Established employer relationships	Often underutilized; limited structured intake

10.2 Differentiation Opportunity

The research identifies a differentiated position for an **AI-first triage-and-match product** targeting the **mid-market employer segment (200–5,000 employees)** with validated clinical intake as the core engine:

- Validated instruments (PHQ-9/GAD-7) provide a clinical foundation that directory-style products lack
- Deterministic, auditable scoring (not LLM-generated) addresses the primary clinical risk of AI health tools
- Mid-market pricing and a B2B SaaS model fills the gap between consumer-facing apps and large-enterprise platforms
- A navigation-and-match design — rather than ongoing support — avoids the regulatory surface area of therapeutic tools

Note: This differentiation analysis is based on desk research and public product information as of 2026. It represents a positioning observation, not a comprehensive competitive audit. Direct competitive analysis with current feature parity mapping should be conducted prior to launch.

11. Key Research Conclusions

The following conclusions are supported by the evidence reviewed and directly inform the product and business decisions documented in the companion Product Dossier:

C1. The scale of unmet need is large and measurable. Approximately 29.5 million adults with mental illness received no treatment in 2024. This is a quantifiable, addressable population, not an amorphous concept.

C2. The primary failure is structural, not motivational. The access barriers documented — wait times, insurance opacity, lack of self-assessment tools, and absent matching logic — are system failures that technology can meaningfully reduce without requiring regulatory or policy change.

C3. The economic case for employer buyers is strong. Combined, absence-related mental health costs (~\$47.6B/year), depression productivity losses (~\$44B/year), and the broader avoidable cost of mental health inequities (~\$477.5B/year) create a clear ROI case for employer-sponsored navigation tools.

C4. AI-enabled interventions have an emerging evidence base. The 2026 RCT by McFadyen et al. and the 2024–2025 PMC comparative study provide meaningful evidence that AI-augmented CBT and digital interventions can achieve outcomes comparable to traditional delivery at lower cost. Crisis detection evidence from Wysa is directional but consistent with the theoretical basis.

C5. The regulatory pathway is navigable with clear scope limits. Positioning a triage-and-navigation tool as a general wellness product (not SaMD), using validated screening instruments without diagnostic claims, and ensuring deterministic (not LLM-generated) clinical scoring represents a defensible and achievable regulatory posture.

C6. A mid-market product gap exists. No currently dominant product serves the 200–5,000-employee employer segment with validated clinical intake, deterministic scoring, and multi-provider routing at accessible price points.

12. References

All references are presented with: publication year, source type, and reliability rating. Where a figure originates from a primary source, it is attributed to that source even if first encountered via a secondary synthesis.

[1] Substance Abuse and Mental Health Services Administration (SAMHSA). (2025). *Key Substance Use and Mental Health Indicators in the United States: Results from the 2024 National Survey on Drug Use and Health (NSDUH)*. Federal survey. **[High]**

<https://www.samhsa.gov/data/sites/default/files/reports/rpt56287/2024-nsduh-annual-national/2024-nsduh-annual-national.htm>

[4] SAMHSA. (2025). *2024 NSDUH Annual National Report (full report)*. Federal survey. **[High]** Primary source for the 61.5 million / 23.4% and treatment-gap figures.

<https://www.samhsa.gov/data/sites/default/files/reports/rpt56287/2024-nsduh-annual-national-report.pdf>

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[9] HRSA, National Center for Health Workforce Analysis. (December 2025). *Behavioral Health Workforce Brief 2025*. Federal agency. **[High]** Primary source for HPSA data (40%/137M, as of December 2, 2025) and psychiatrist count (52,164 in 2023). <https://bhw.hrsa.gov/sites/default/files/bureau-health-workforce/data-research/Behavioral-Health-Workforce-Brief-2025.pdf>

[10] Commonwealth Fund. (November 21, 2024). *State of Health Insurance Coverage in the U.S.: 2024 Biennial Survey*. Nonpartisan policy institute. **[High]** <https://www.commonwealthfund.org/publications/surveys/2024/nov/state-health-insurance-coverage-us-2024-biennial-survey>

[11] Commonwealth Fund. (August 5, 2024). *Enforcing Mental Health Parity: State Options to Improve Access to Care*. Policy brief; qualitative 10-state sample. **[Moderate-High]** <https://www.commonwealthfund.org/publications/issue-briefs/enforcing-mental-health-parity-state-options-improve-access-care>

[13] Parity-tracking synthesis (ParityTrack / KFF / Commonwealth Fund / DOL). (2026). *Mental Health Parity Laws by State*. Multi-source commercial synthesis. **[Moderate]** <https://www.moneygeek.com/insurance/health/mental-health-parity/>

[14] Commonwealth Fund. (May 25, 2023). *Building on Behavioral Health Parity: State Options to Strengthen Access*. Nonpartisan policy. **[High]** <https://www.commonwealthfund.org/blog/2023/building-behavioral-health-parity-state-options-strengthen-access-care>

[15] NAMI survey data (via insurance/MHPAEA analysis). (2024). *Mental Health Parity Update*. Advocacy survey. **[Moderate]** Methodology not fully detailed; exact ratio varies by source. <https://w3ins.com/news/mental-health-parity-update-2024/>

[16] National Council for Mental Wellbeing / MTM Services. *CCBHC Impact Report*. MTM Services analysis of ~10,000 care-access flowcharts from ~1,000 centers across 47 states, 2021. **[Moderate-High]** Specific to community mental health centers; not a universal behavioral health average. <https://www.thenationalcouncil.org/eliminate-barriers-to-access/>

[18] Subotic-Kerry, M. et al. (2025). *While they wait: a cross-sectional survey on wait times for mental health services*. *BMJ Open*, 15(3), e087342. Peer-reviewed. **[High]** Limitation: international sample. <https://bmjopen.bmj.com/content/15/3/e087342>

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Limitation: self-selected respondents. <https://www.npr.org/sections/health-shots/2023/12/06/1217487323/psychologists-waitlist-demand-mental-health-care>
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Limitation: five-state, 2022 data.
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